Bupa IT Strategy Proposal Outline

1. Working Title

One Bupa Technology Strategy: Safe, Fast, Connected

A meta-strategy for IT decisions that translates Bupa's customer-centric healthcare ambition into architecture, engineering, platform, and delivery choices.

2. Core Proposition

Bupa should organise its next phase of IT strategy around one idea:

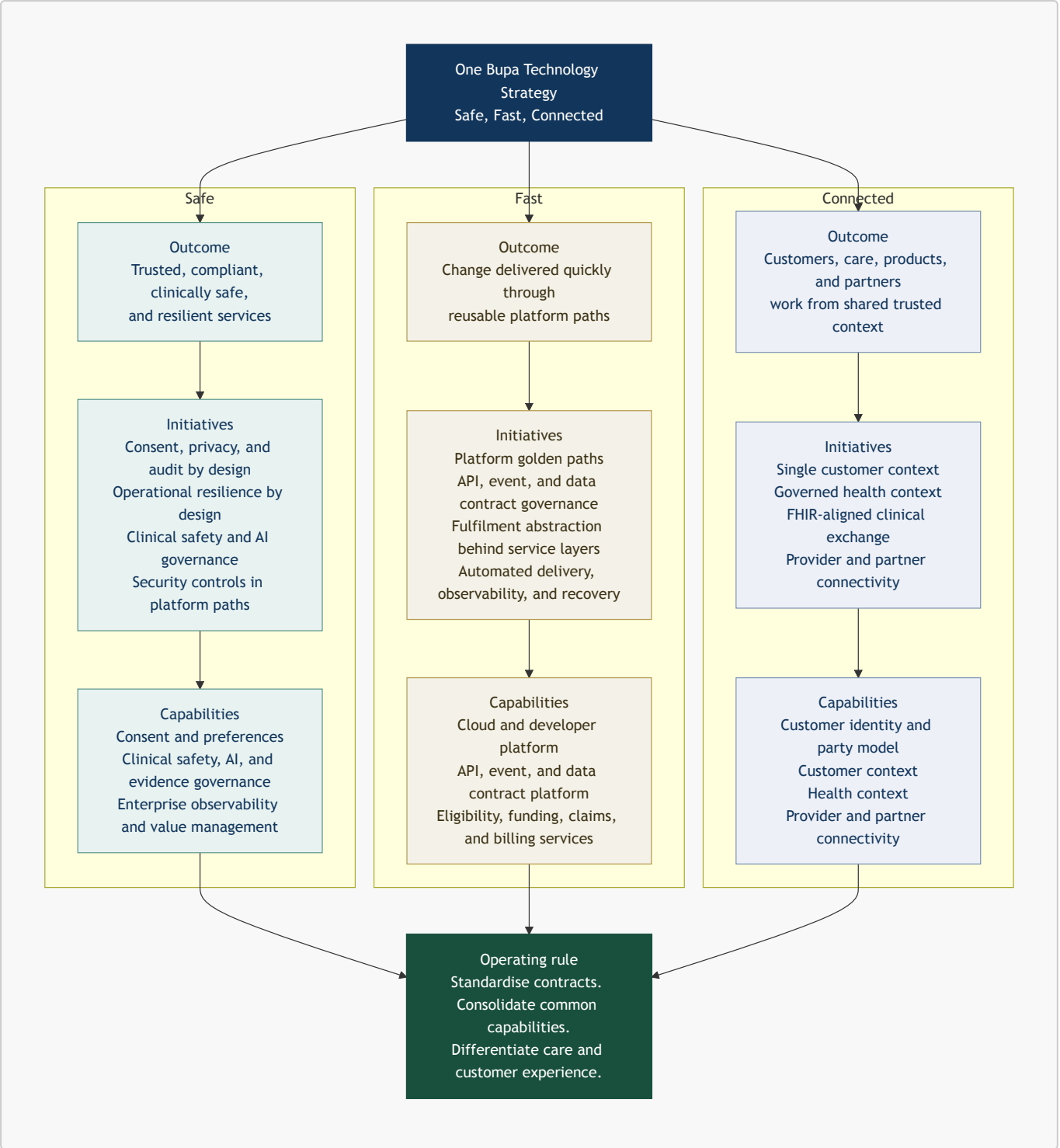
Create a governed connected health and care technology platform that gives Bupa trusted customer and health context, interoperable care and insurance services, and safe delivery paths by default.

The purpose is to make customer, clinical, and cost outcomes easier to improve and measure across markets.

The strategy is not another enterprise architecture diagram. It is a decision framework for where to consolidate, where to standardise, where to abstract local complexity, where resilience and clinical safety must be mandatory, and where business units should continue to differentiate.

3. Safe, Fast, Connected Strategy View

This view assigns the initial initiatives and enterprise capabilities to the three strategy areas.



4. Strategic Shifts

Strategic Shift	From	To	Decision It Drives
1. From fragmented context to trusted context	Customer, health, consent, product, and journey data held in separate systems	Shared, governed views of customer, health, consent, eligibility, and interaction history	Stop rebuilding identity, customer, and health context in every channel or business unit
2. From point-to-point integration to	Local integrations, product-specific APIs, duplicated data flows	Standard APIs, events, data contracts, clinical protocols, and	Standardise on contracts and

Strategic Shift	From	To	Decision It Drives
enterprise interoperability		interoperability patterns	protocols before products
3. From local delivery effort to platform-enabled delivery	Teams repeatedly solving security, compliance, deployment, observability, resilience, and cloud foundations	Platforms that make safe, fast, resilient, efficient delivery the default path	Shift effort from repeated plumbing to product, care, and customer outcomes

5. Priority Enterprise Capabilities

The first capability footprint should be small enough to govern and broad enough to change delivery behaviour.

Capability	Enterprise Purpose
Customer identity and party model	Establish one trusted representation of customers, members, patients, brokers, providers, clinicians, and related parties.
Consent and preferences	Separate clinical consent, privacy permissions, marketing preferences, data-sharing permissions, and audit evidence.
Customer context	Provide reusable views of relationship, coverage, eligibility, product, interaction, and journey history.
Health context	Provide governed access to relevant health information, clinical context, care plans, observations, and care history.
Eligibility, funding, claims, and billing services	Abstract insurance and fulfilment complexity behind reusable service layers.
Provider and partner connectivity	Make referrals, appointments, directories, claims, documents, and data exchange easier to integrate across the ecosystem.
API, event, and data contract platform	Manage reusable service contracts, event schemas, data products, versioning, conformance, and lifecycle controls.
Cloud and developer platform	Provide golden paths for secure delivery, deployment, observability, recovery, cost management, and compliance.
Clinical safety, AI, and evidence governance	Ensure digital, data, automation, and AI-enabled services are clinically appropriate, auditable, and responsibly operated.
Enterprise observability and value management	Make reliability, adoption, journey performance, unit cost, safety, and value visible enough to manage.

6. Decision Rights and Ownership

Each enterprise capability needs one accountable owner with authority over roadmap, funding, standards, adoption, lifecycle health, and value evidence.

Decision Area	Default Decision Right
Capability ownership	Enterprise capability owner, with market and business unit representation.
Architecture standards	Enterprise architecture, platform, security, privacy, data, and clinical safety authorities.
Product and platform roadmaps	Capability owner and platform/product leadership, tied to funded outcomes.
Local variation	Market or business unit owner, approved through the exception model.
Retirement and convergence	Capability owner and investment governance, with explicit transition funding.
Risk acceptance	Named accountable executive, supported by security, privacy, resilience, and clinical safety evidence.

7. Mandatory Standards and Constraints

The strategy should make a small set of standards mandatory wherever the relevant pattern applies.

Standard or Constraint	Where It Applies
Contracts before products	New integrations, platform choices, data products, ecosystem services, and major procurements.
FHIR-aligned clinical exchange	Clinical and health data interoperability, where the use case involves health records, observations, care plans, eligibility, claims, consent, provenance, or audit events.
OpenAPI and AsyncAPI contract governance	Synchronous APIs, asynchronous events, partner interfaces, and internal service boundaries.
Data product controls	Critical customer, health, operational, and financial data used for decisions, AI, analytics, or reporting.
Consent, privacy, and audit by design	Customer, health, marketing, personalisation, AI, and ecosystem data-sharing use cases.
Operational resilience by design	Important business services, clinical workflows, customer journeys, payment/claims flows, and critical platform dependencies.
Cloud-ready and portable architecture	New services, material platform changes, and strategic vendor decisions.
Platform golden paths	Delivery pipelines, observability, security controls, deployment, recovery, and cost management.

8. Proposed Principles

1. Customer, Clinical, Cost is the primary trade-off model.
2. One customer, many journeys.

3. One health view, governed by consent, privacy, clinical safety, and audit evidence.
4. Contracts before products.
5. Abstract fulfilment complexity behind service layers.
6. Cloud-ready and portable by design.
7. Platforms make the right path easy and the risky path visible.
8. Consolidate where Bupa needs leverage.
9. Differentiate where local market value matters.
10. Measure reuse, speed, safety, efficiency, and outcomes.

9. Outcome Scorecard

The strategy should be judged by whether it changes delivery behaviour and business outcomes, not by whether the target architecture is documented.

Outcome	Example Measures
Reuse	Adoption of shared capabilities, API/event reuse, duplicated system retirement, percentage of journeys using enterprise context services.
Speed	Lead time for change, deployment frequency, onboarding time for new services or partners, time to launch market variation.
Safety	Clinical safety evidence, policy compliance, privacy incidents, security incidents, high-severity defects, resilience test outcomes.
Efficiency	Cost-to-serve, platform unit cost, operational effort removed, manual work reduced, integration run cost reduced.
Customer and care outcomes	Journey completion, digital containment where appropriate, NPS-linked journey improvements, care continuity, claims/payment friction.
Data quality and trust	Data product quality scores, lineage coverage, consent traceability, decision-grade dataset adoption.

10. Exception Model

Local differentiation is allowed where it creates market value, responds to regulation, or protects a clinically necessary workflow. It should not be an unmanaged default.

Each exception should have:

1. A named owner.
2. The customer, clinical, cost, regulatory, or market reason for variation.
3. The cost and risk of divergence.
4. A review date or expiry condition.
5. A convergence, containment, or deliberate-local decision.

11. Updated One-Page Message

One Bupa Technology Strategy: Safe, Fast, Connected

To become the world's most customer-centric healthcare company, Bupa needs technology that can create trusted customer and health context, connect care safely, and deliver change quickly across markets.

The strategy is built around three strategic shifts:

1. From fragmented context to trusted context.
2. From point-to-point integration to enterprise interoperability.
3. From local delivery effort to platform-enabled delivery.

The rule is simple:

Standardise the contracts. Consolidate the common capabilities. Differentiate the care and customer experience.

The decision test is equally simple:

If an initiative does not improve customer, clinical, or cost outcomes; reuse enterprise capabilities; and make delivery safer, faster, or more efficient, it should not be treated as strategic.